

CLINICAL INTELLIGENCE TREND REPORT

RED: Provider Review Recommended

Patient ID: Wheeler

Period: April 1 to April 30, 2026

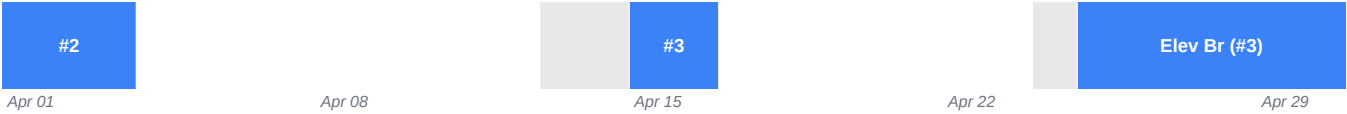
Report Date: May 1, 2026

Coverage: 240/720h (33%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 30 days from Apr 01 to Apr 30

■ HR ■ Breathing



Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
28	92h	Elevated Breathing	<1	33%

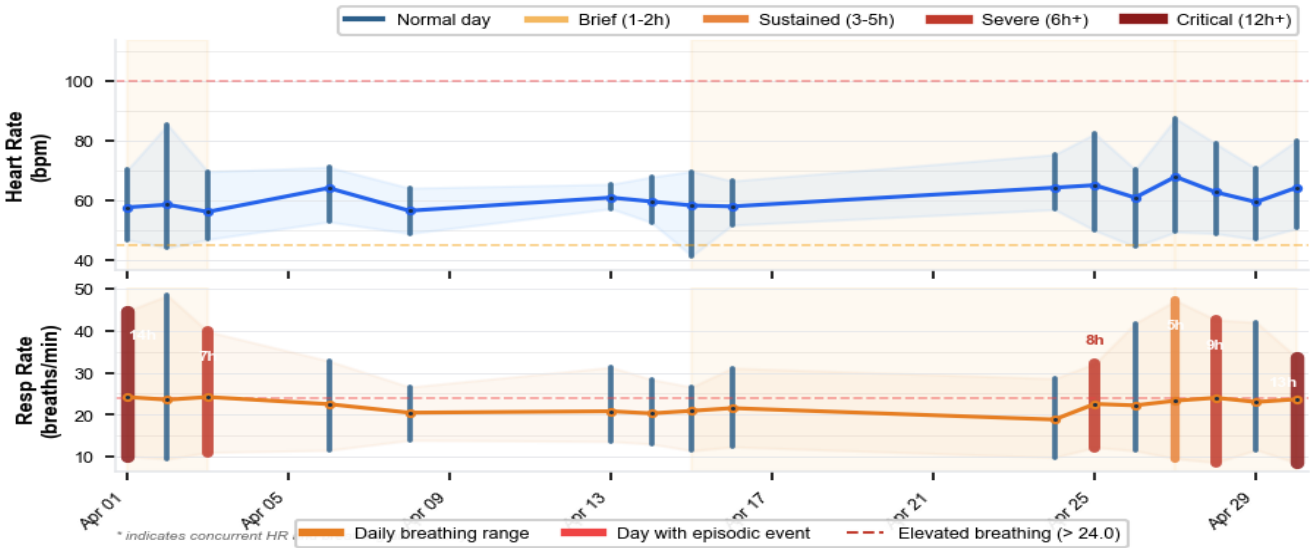
Major Findings: Sustained elevated breathing (avg 26, peak 48)

#	Episode	Min/Max	Total Hours	Longest Continuous	Episodes/day	Average	Date
1	High Breathing	33 brpm	1h	1h	1	30 brpm	Apr 27
2	Elevated Breathing	48 brpm	32h	14h	3	26 brpm	Apr 01 to Apr 03
3	Elevated Breathing	47 brpm	59h	13h	1	26 brpm	Apr 15 to Apr 30

Low data coverage (33%); interpret with caution.

Clinical Pattern Observations:

- Sustained finding:** 14h continuous Elevated Breathing on Apr 01.
- Clustered pattern:** Episodic events on 6 of 30 days (20%).
- Nocturnal pattern:** 3 of 4 eligible episodes during evening or night hours.



Red bars indicate days with detected episodic events.

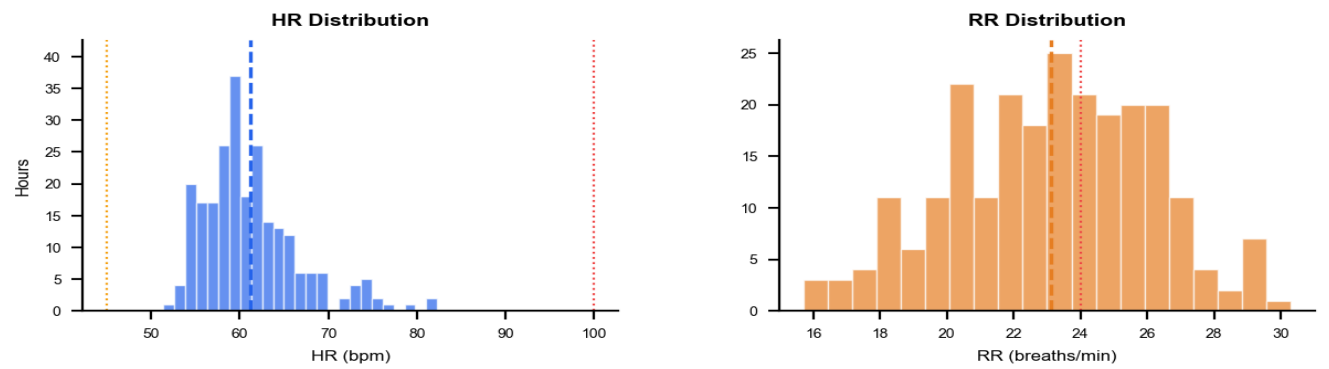
■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

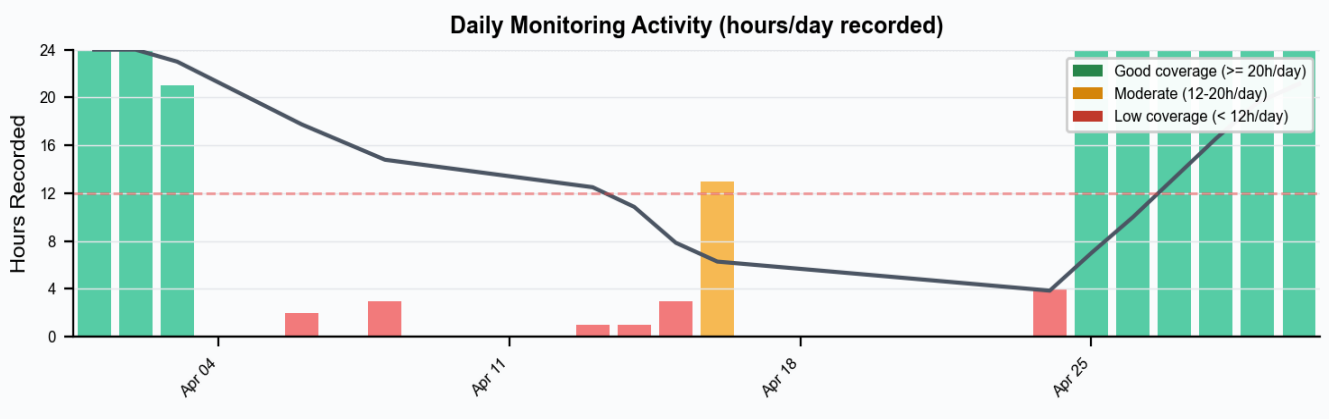
Very Low HR < 40 bpm	Low HR < 45 bpm	Elevated HR > 95 bpm	High HR > 100 bpm
Very High HR > 110 bpm	Elevated Breathing > 24 brpm	High Breathing > 30 brpm	Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.